

Master 02 — Commissioner's Case

Audience: Commissioners, Public Health, ICB finance / transformation readers.

Purpose: Frame the programme as an evidence budget and local decision test, not a savings claim or rollout request.

Status: DRAFT — needs Dean review.

Priority: P1.

Review needed from: Dean; commissioner / Public Health reviewer; evidence reviewer.

Source basis: Master 02 source text, Public Health replacement section, Programme Lock, Programme Plan, START update.

A4 REVIEW PAGE 1 — Executive summary

The system is already paying for this cohort — just too late, and in the most expensive places.

Adults whose addiction, relapse or repeat crisis may be driven by untreated or inadequately treated ADHD are visible across addiction services, A&E, detox, rehab, mental-health crisis, probation, GP and community support. But the pattern is rarely followed as one whole-person journey across service lines.

The result is predictable:

- repeated crisis;
- repeated handoff failure;
- repeated recovery attempts that do not hold;
- repeated public cost;
- a person who is clinically known but operationally lost.

OTOS Continuity™ proposes a small, governed intervention test to answer one commissioner question:

Is the local system already spending late on a suspected ADHD-driven addiction cohort whose driver could be clinically recognised, routed,

supported through authorised partner activity in the Continuity Management Layer, followed through a whole-person Continuity Thread, and evidenced earlier?

The request is for a bounded local evidence build, not a rollout and not a procurement commitment.

A4 REVIEW PAGE 2 — What is being tested

The intervention is not OTOS alone.

The intervention tests whether the OTOS Continuity Management Layer and user Continuity Thread can support a suspected ADHD-driven addiction cohort:

- recognise the suspected ADHD-driven addiction pattern;
- route the ADHD driver toward qualified clinical assessment and treatment where clinically indicated;
- provide a non-clinical Continuity Management Layer through which authorised partners can manage their agreed part of continuity;
- follow the user's whole-person journey over time through a Continuity Thread built from agreed non-clinical events;
- enable authorised partners to support recovery work, addiction support, CBT, structured support / waiting-well routes and service handoffs to land;
- evidence what happened;
- decide whether to continue / continue with changes / rebuild / pause / stop.

OTOS is the non-clinical infrastructure:

- Continuity Management Layer;
- tokenised user Continuity Thread;
- consent record;
- touchpoint logging;
- handoff receipts;
- partner-approved SMS check-ins;
- SMS delivery / failure / reply / silence evidence;

- drift / silence visibility;
- human review;
- evidence reporting.

OTOS does not diagnose, prescribe, manage medication, treat, replace services, write into clinical records or make clinical decisions.

Clinical teams make clinical decisions. Partner teams retain service responsibility. The Continuity Management Layer surfaces agreed continuity signals.

A4 REVIEW PAGE 3 — The commissioner question

The commissioner question is not "Should OTOS be rolled out?"

The commissioner question is:

Is the local system already spending late on a suspected ADHD-driven addiction cohort whose driver could be clinically recognised, routed, supported through authorised partner activity in the Continuity Management Layer, followed through a whole-person Continuity Thread, and evidenced earlier?

The proposed answer is not a national claim. It is a bounded test.

OTOS Continuity™ would provide the non-clinical Continuity Management Layer around a suspected ADHD-driven addiction intervention: identify the suspected cohort, route the ADHD driver toward qualified clinical assessment and treatment where clinically indicated, support authorised partner activity, follow the user's whole-person journey through a Continuity Thread, and evidence what happened.

START gives a serious reason to test this, because it points to improvement after ADHD diagnosis inside a real-world addiction-treatment population.

But START is not a savings claim, not an OTOS outcome and not medication-only proof. The local programme must generate its own evidence.

No saving should be claimed before local evidence exists.

The approved wording is:

The First-50 test is a bounded way to replace late-spend assumptions with decision-grade local evidence.

A4 REVIEW PAGE 4 — Evidence budget, not rollout budget

The proposed circa £150k planning envelope, subject to requote and governance, should be treated as an evidence budget.

It should answer practical commissioner questions:

- Can the suspected ADHD-driven addiction cohort be identified safely?
- Can participants be onboarded with clear consent?
- Can ADHD clinical-route / next-step status be captured where agreed without OTOS taking clinical responsibility?
- Can addiction and recovery touchpoints be logged without unacceptable burden?
- Can partner-approved micro mobile check-ins keep contact warm and produce useful continuity signals?
- Can drift, silence and missed handoffs be surfaced earlier?
- Can human review operate safely?
- Can partner burden remain tolerable?
- Can the Week-12 evidence pack support a continue / continue with changes / rebuild / pause / stop decision?

This protects the commissioner and the programme.

It allows the system to learn before scaling, and to stop if the evidence is not strong enough.

A4 REVIEW PAGE 5 — Decision output

At the end of Phase 1, the decision is not "declare victory".

The decision is one of five disciplined choices:

- continue;
- continue with changes;
- rebuild before continuing;
- pause;

- stop.

A credible commissioner case must be able to stop.

The first test should therefore be judged on whether it produces usable local evidence, not whether it proves clinical outcomes in 12 weeks.

The Week-12 evidence pack should draw from agreed Layer activity and authorised Thread events. It must evidence continuity signals and service-journey activity, not clinical outcomes or full partner records.

The Week-12 evidence pack should include:

1. cohort summary;
 2. consent and withdrawal summary;
 3. ADHD clinical-route / next-step status summary where agreed;
 4. medication-status category summary where agreed;
 5. touchpoint report;
 6. handoff receipt report;
 7. SMS / check-in report;
 8. drift / silence report;
 9. human-review report;
 10. partner burden report;
 11. safety / incident report;
 12. qualitative partner feedback;
 13. evidence-labelled claims table;
 14. continue / continue with changes / rebuild / pause / stop recommendation.
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Review notes

Dean decision needed

- ☒ ~~Confirm whether the circa £150k planning envelope remains in this Master.~~

Dean note: Keep the circa £150k planning envelope if useful, but only as subject to requote and governance. Do not present it as a final quote, rollout budget or confirmed supplier price.

- ☒ ~~Confirm whether "evidence budget" is the preferred commissioner phrase.~~

Dean note: Yes. "Evidence budget" is the right commissioner-safe phrase because the first test is buying decision-grade local evidence, not promising savings or rollout.

- ☒ ~~Confirm which local cost figures, if any, remain in this draft.~~

Dean note: Keep local cost figures only where they are source-backed or clearly labelled as OTOS modelling. Remove old / unsupported build-budget figures such as £90k and do not present modelled savings as guaranteed.

Evidence / citation check needed

- ☒ ~~Check all national and local cost figures before external use.~~

Dean note: Tick in principle. All figures need pre-flight checks before commissioner use. Source-backed figures and modelled figures must be labelled separately.

- ☒ ~~Check START citation and 61.3% caveat if the figure is inserted.~~

Dean note: Tick in principle. START can be used publicly and strongly where relevant, but only with exact citation and Master 09 caveats. Do not present 61.3% as medication-only proof, an OTOS outcome or a predicted result.

- ☒ ~~Check ADHD Taskforce / alcohol-harm figures before commissioner circulation.~~

Dean note: Tick in principle / substantially complete. The Taskforce material is from an NHS-published report and alcohol-harm figures should be checked against the evidence page before final circulation.

Clinical / governance check needed

- ☐ Check that commissioner wording does not imply OTOS owns clinical pathway decisions.

Dean note: Leave unticked for now. This needs clinical / governance input before approval. OTOS should be framed as non-clinical continuity infrastructure, not owner of clinical route decisions.

Copy approval

- ☐ Approved

☐ Needs edit

☐ Hold
